Ivosidenib (Tibsovo®) and azacitidine

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What are ivosidenib (Tibsovo®) and azacitidne?

Ivosidenib (Tibsovo®) is a type of targeted therapy drug called a cancer growth inhibitor. Azacitidine is a chemotherapy drug.

Ivosidenib and azacitidine treatment is used to treat acute myeloid leukaemia (AML). You may be given this treatment if the cancer has a change (mutation) in a gene called IDH1. Your doctor arranges for tests on the leukaemia cells to check for this change before you begin treatment.

It is best to read this information with our general information about targeted therapy drugs, chemotherapy and the type of cancer you have.

Your cancer team will talk to you about this treatment and its possible side effects before you agree (consent) to have treatment.

More information about this treatment

This information is correct at time of publishing. But sometimes the types of cancer this treatment is used for, or treatment side effects, may change between revision dates.

You can talk to your cancer team if you want more detailed information about this treatment. Or visit the electronic Medicines Compendium (eMC) website, which has patient information leaflets (PIL) for individual drugs.

How ivosidenib and azacitidine are given

You usually have azacitidine in a chemotherapy day unit or clinic as an outpatient.

During a course of treatment, you will meet someone from your cancer team, such as a:

- blood cancer doctor (haematologist)

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- specialist nurse
- specialist pharmacist.

This is who we mean when we mention doctor, nurse or pharmacist in this information.

Before or on the day of each cycle of treatment, you will have a blood test. This is to check that it is safe for you to have treatment. You may also have regular blood tests during your treatment.

You will also have an ECG (electrocardiogram). This checks the electrical activity of your heart. It can show any changes to how your heart is working. You have an ECG before you start taking ivosidenib. You then have an ECG every week for the first 3 weeks of treatment. After this, some people have an ECG every month during their treatment. Your cancer doctor, nurse of pharmacist will explain how often they will be done.

You will meet with a doctor, nurse or pharmacist before you have treatment. They will talk to you about your blood results and ask how you have been feeling. If your blood results are okay, the pharmacy team will prepare your cancer drugs.

You usually have azacitidine as an injection under the skin (subcutaneous). The nurse will give it to you. It may be into the skin on your thigh, tummy or upper arm. Your nurse will give you anti-sickness drugs before the azacitidine.

You have ivosidenib as tablets that you take at home. Your doctor, nurse or pharmacist will explain how to take your tablets.

Taking ivosidenib tablets

You take ivosidenib once a day. Your doctor, nurse or pharmacist will tell you how many tablets to take.

Make sure you:

- take them on an empty stomach – do not eat anything for 2 hours before taking the tablets and for 1 hour after
- swallow them whole with a glass of water
- do not chew, break or crush them
- take them at the same time every day
- avoid grapefruit and grapefruit juice.

If you forget to take the tablets, take the missed dose as soon as possible if it is within 12 hours of the missed dose. If more than 12 hours have passed, do not take this dose. Take your next dose at your usual time.

Do not take a double dose. Tell your doctor, nurse or pharmacist at your next appointment that you missed a tablet.

If you are sick (vomit) just after taking ivosidenib, do not take an extra dose. Take your next dose at the usual time. Do not take a double dose.

Other things to remember about your tablets:

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Keep them in the original package and at room temperature, away from moisture, heat and direct sunlight.

- Keep them safe, where children cannot see or reach them.
- Do not throw away unused tablets. Return them to your cancer team at the hospital.
- Do not eat the sachet found in the bottle.

Your course of treatment

Your cancer team will discuss your treatment plan with you. They may give you a copy of the plan to take home.

Each cycle of treatment lasts 28 days (4 weeks). You have:

- azacitidine injections once a day for 7 days, followed by 21 days rest
- ivosidenib as tablets once a day, every day

Your doctor, nurse of pharmacist will give you the ivosidenib tablets to take home. Always take them exactly as they tell you to. This is important to make sure they work as well as possible for you.

You continue taking this treatment as long as they are working for you and any side effects can be managed. Your cancer team may also give you anti-sickness drugs and other medicines to take home. Take all your capsules or tablets exactly as they tell you to.

Side effects while treatment is being given

Some people may have side effects while they are having a cancer drug or shortly after they have it:

Allergic reaction

Some people have an allergic reaction while having this treatment. Signs of a reaction can include:

- feeling hot or flushed
- shivering
- itching
- a skin rash
- feeling dizzy or sick
- a headache
- feeling breathless or wheezy
- swelling of your face or mouth
- pain in your back, tummy or chest.

Your nurse will check you for signs of a reaction during your treatment. If you feel unwell or have any of these signs, tell them straight away. If you do have a reaction, they can treat it quickly.

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Sometimes a reaction happens a few hours after treatment. If you develop any of these signs or feel unwell after you get home, contact the hospital straight away on the 24-hour number.

Always call 999 if swelling happens suddenly or you are struggling to breathe.

Problems at the injection site

The skin in the area that was injected may become:

- painful
- swollen
- red, if you have white skin
- darker, if you have black or brown skin.

If this happens, tell your doctor or nurse. Your nurse will give you the injection into different places each time to help avoid this.

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About side effects

We explain the most common side effects of this treatment here. We also include some that are less common.

You may get some of the side effects we mention, but you are unlikely to get all of them. And you may have some side effects, including rarer ones, that we have not listed here.

Always tell your doctor, nurse or pharmacist about any side effects you have. They can give you:

- drugs to help control some side effects
- advice about managing side effects.

It is important to take any drugs exactly as explained. This means they will be more likely to work for you.

Serious and life-threatening side effects

Some cancer treatments can cause serious side effects. Sometimes, these may be life-threatening. Your doctor, nurse or pharmacist can explain the risk of these side effects to you.

Contact the hospital

Your doctor, nurse or pharmacist will give you 24-hour contact numbers for the hospital. If you feel unwell or need advice, you can call at any time of the day or night. Save these numbers in your phone or keep them somewhere safe.

Your patient alert card

They will also give you a card with information about your treatment. It is important to keep the card with you during treatment and after treatment ends. You should show it to any doctor or healthcare professional caring for you.

Very common side effects

These side effects happen to 10 or more people in every 100 people (10% or more) who have this treatment.

Differentiation syndrome in AML

Differentiation syndrome is the body’s response to certain types of anti-cancer drugs. This can cause large amounts of immune substances called cytokines to be released quickly into the blood from the leukaemia cells. This needs treatment straight away.

If you notice any of the side effects below, contact the hospital straight away on the 24-hour number:

- high temperature (fever)
- breathlessness

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- cough
- rash
- passing less urine (pee)
- feeling dizzy or light-headed
- gaining weight quickly
- swelling of your arms or legs.

You will be given an alert card with the drug. Take it with you at all times. It has important information about the symptoms of differentiation syndrome and what you should do if you have any symptoms.

Risk of infection

This treatment can reduce the number of white blood cells in your blood. These cells fight infection. If your white blood cell count is low, you may be more likely to get an infection. A low white blood cell count is called neutropenia.

An infection can be very serious when the number of white blood cells is low. It is important to get any infection treated as soon as possible. If you have any of the following symptoms, contact the hospital straight away on the 24-hour number:

- a temperature above 37.5°C
- a temperature below 36°C
- you feel unwell, even with a normal temperature
- you have symptoms of an infection.

Symptoms of an infection include:

- feeling shivery and shaking
- a sore throat
- a cough
- breathlessness
- diarrhoea
- needing to pass urine (pee) often, or discomfort when you pass urine.

It is important to follow any specific advice your cancer treatment team gives you.

Your white blood cell count will usually return to normal before your next treatment. You will have a blood test before having more treatment. If your white blood cell count is low, your doctor may delay your treatment for a short time, until your cell count increases.

Bruising and bleeding

This treatment can reduce the number of platelets in your blood. Platelets are cells that help the blood to clot.

If the number of platelets is low, you may bruise or bleed easily. You may have:

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nosebleeds or bleeding gums

- bleeding gums
- heavy periods
- blood in your urine (pee) or stools (poo)
- tiny red, brown or purple spots that may look like a rash – these spots can be harder to see if you have black or brown skin
- red area in the white of the eyes.

If you have any unexplained bruising or bleeding, contact the hospital straight away on the 24-hour number. You may need a drip to give you extra platelets. This is called a platelet transfusion.

Anaemia (low number of red blood cells)

This treatment can reduce the number of red blood cells in your blood. Red blood cells carry oxygen around the body. If the number of red blood cells is low, this is called anaemia. You may feel:

- very low in energy
- breathless
- dizzy and light-headed.

If you have these symptoms, contact the hospital straight away on the 24-hour number. You may need treatment for anaemia. If you are very anaemic, you may need a drip to give you extra red blood cells. This is called a blood transfusion.

Feeling sick

Your doctor, nurse or pharmacist will prescribe anti-sickness drugs to help prevent or control sickness. Take the drugs exactly as they tell you to, even if you do not feel sick. It is easier to prevent sickness than to treat it after it has started.

If you feel sick, take small sips of fluid often and eat small amounts regularly. It is important to drink enough fluids. If you continue to feel sick, or if you are sick (vomit) 1 to 2 times in 24 hours, contact the hospital on the 24-hour number as soon as possible. They will give you advice. They may change your anti-sickness treatment. Let them know if you still feel sick.

Effects on the heart

This treatment can affect how the heart works. It may also cause fluid to build up around the heart. Less commonly, it can cause swelling around the heart.

You will have tests to check how well your heart is working. These may be done before, during and after treatment.

If the treatment is causing heart problems, your doctor may change the type of treatment you are having.

Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- breathlessness

- dizziness
- changes to your heartbeat
- swollen feet and ankles
- pain or tightness in your chest that is worse when you breathe in
- a high temperature, cough or flu like symptoms.

Other conditions can cause these symptoms, but it is important to get them checked by a doctor.

Always call 999 if you have:

- chest pain, pressure, heaviness, tightness or squeezing across the chest
- difficulty breathing.

Diarrhoea

This treatment may cause diarrhoea. Diarrhoea means passing more stools (poo) than is normal for you, or having watery or loose stools. You may also have stomach cramps. If you have a stoma, it may be more active than usual.

If you are passing loose stools 3 or more times a day and this is not normal for you, contact the hospital as soon as possible on the 24-hour number. Follow the advice they give you about:

- taking anti-diarrhoea medicines
- drinking enough fluids to keep you hydrated and to replace lost salts and minerals
- any changes to your diet that might help.

They might also ask you for a specimen of your stool to check for infection.

Constipation

This treatment can cause constipation. Constipation means that you are not able to pass stools (poo) as often as you normally do. It can become difficult or painful. Here are some tips that may help:

- Drink at least 2 litres (3½ pints) of fluids each day.
- Eat high-fibre foods, such as fruit, vegetables and wholemeal bread.
- Do regular gentle exercise, like going for short walks.

If you have constipation, contact the hospital on the 24-hour number for advice. They can give you drugs called laxatives to help.

If you have not been able to pass stools for over 2 days and are being sick, contact the 24-hour number straight away.

Tummy pain

You may get pain in your tummy (abdomen), or have indigestion. Your doctor, nurse or pharmacist can give you advice or treatment to help. Contact the hospital straight away on the 24-hour number if your symptoms do not go away, or get worse.

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Effects on the lungs

This treatment can cause changes to the lungs. Tell your doctor, nurse or pharmacist if you develop:

- a cough that does not go away
- wheezing
- breathlessness.

You should also tell them if any existing breathing problems get worse. You may have tests to check your lungs.

Headaches

This treatment may cause headaches. If you have headaches, tell your doctor, nurse or pharmacist. They can give you advice about painkillers that may help. Tell them if the headache does not get better, or gets worse.

If you develop a sudden or severe headache, or feel confused contact the hospital on the 24-hour straight away.

Feeling dizzy

This treatment can cause dizziness. Tell your doctor or nurse if this is a problem. It is important not to drive or operate machinery if you notice these effects.

Muscle or joint pain

You may get pain in your muscles or joints for a few days after treatment. If this happens, tell your doctor, nurse or pharmacist. They can give you painkillers and advice. They can also tell you if any of the painkillers you usually take are suitable.

Tell them if the pain does not get better. Having warm baths and resting regularly may help.

Skin changes

This treatment can affect your skin. It might feel dry. You may develop a rash, which may be itchy. Always tell your doctor, nurse or pharmacist about any skin changes. They can give you advice or prescribe creams or medicines to help.

If your skin feels dry, try using soap-free cleansers and unperfumed moisturising cream every day.

Very rarely, this treatment can cause sores in the skin which can be serious. If you notice sores or broken areas of skin, tell your doctor or nurse straight away.

Low blood potassium

Azacitidine can cause low levels of potassium in the blood. You will have regular blood tests to check your potassium levels.

If your levels are very low, you may need treatment and your doctor may tell you to stop taking azacitidine. Symptoms of low potassium include:

- weakness
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twitching muscles

- feeling that you heart is beating harder or faster than usual.

If you notice these symptoms, contact the hospital straight away on the 24-hour number.

Loss of appetite

This treatment can affect your appetite. Don’t worry if you do not eat much for 1 or 2 days. But if your appetite does not come back after a few days, or if you are losing weight, tell your doctor, nurse or pharmacist. They can give you advice. They may give you food or drink supplements. Or they may suggest changes to your diet or eating habits to help.

Difficulty sleeping (insomnia)

Sometimes this treatment can affect your sleep. Tell your doctor, nurse or pharmacist if cancer treatment makes it difficult to sleep.

Feeling tired

Feeling tired is a common side effect of this treatment. It is often worse towards the end of treatment and for some weeks after it ends. Try to pace yourself and plan your day so you have time to rest. Gentle exercise, like short walks, can help you feel less tired.

If you feel sleepy, do not drive or use machinery.

Other side effects

These side effects happen to less than 10 in 100 people (less than 10%) who have this treatment. Some of them are much rarer than this but they are still important to know about. Rare means a side effect that happens to less than 1 in 1,000 people (less than 0.1%).

Numb or tingling hands or feet (peripheral neuropathy)

This treatment may affect the nerves in your fingers and toes. This can cause numbness, tingling or pain in your hands or feet. This is called peripheral neuropathy. You might find it hard to do fiddly tasks such as fastening buttons or tying shoelaces.

If you have these symptoms, always tell your doctor, nurse or pharmacist. They sometimes need to change the drug or the dose of the drug. The symptoms usually improve slowly after treatment ends. But for some people they continue and are a long-term side effect of treatment.

Sore mouth and throat

This treatment may cause a sore mouth and throat. You may also get mouth ulcers. This can make you more likely to get a mouth or throat infection. Use a soft toothbrush to clean your teeth or dentures in the morning, at night and after meals.

Contact the hospital straight away on the 24-hour number, if:

- a sore mouth or throat affects how much you can drink or eat

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- your mouth, tongue, throat or lips have any blisters, ulcers or white patches.

They can give you advice, and mouthwash or medicines to help with the pain or to treat any infection. Follow their advice and make sure you:

- drink plenty of fluids
- avoid alcohol and tobacco
- avoid food or drinks that irritate your mouth and throat.

Effects on the kidneys

This treatment can affect how your kidneys or liver work. This is usually mild and goes back to normal after treatment ends. You will have blood tests to check how well your kidneys are working. Contact the hospital on the 24-hour number if you:

- have blood in your urine (pee)
- are passing less urine or peeing less often than usual.

Drinking fluids also helps protect your kidneys. The advice is usually to try to drink at least 2 litres (3½ pints) of fluid each day. But follow any advice from your doctor, nurse or pharmacist about how much is right for you.

Effects on the liver

This treatment can affect how the liver works. You will have regular blood tests to check this. Sometimes liver changes can be serious. Contact the hospital straight away on the 24-hour number if you have any of these symptoms:

- yellow skin or eyes
- feeling very sleepy
- dark urine (pee)
- unexplained bleeding or bruising
- pain in the right side of your tummy (abdomen).

If you have black or brown skin, changes in your skin colour may be less noticeable. You might first notice the whites of your eyes look yellow, or that the colour of your urine has changed.

Bleeding in the eye or in the brain

This drug might cause bleeding in the eye or a bleed in the brain. Contact the hospital on the 24-hour number you have been given if you have:

- broken blood vessels or redness in your eye
- changes to your vision.

Contact the hospital on the 24-hour number you have been given straight away or call 999 if you have:

- a severe headache
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weakness in your body

- changes to your speech.

Changes to blood pressure

This treatment can cause changes in your blood pressure. If you already have problems with your blood pressure, tell your doctor, nurse or pharmacist. You may need to have regular blood pressure checks.

Tumour lysis syndrome (TLS)

Some people are at risk of developing a condition called tumour lysis syndrome (TLS). The risk is highest at the beginning of treatment. TLS happens when treatment makes large numbers of cancer cells die and break down quickly. This releases lots of waste products into the blood and can affect the kidneys and heart.

TLS can be prevented or treated. You will have regular blood tests to check for TLS. You may have:

- extra fluids through a drip
- medicines such as allopurinol tablets or rasburicase through a drip.

Drinking at least 2 litres (3½ pints) of fluid a day will also help.

Falls

You may be more likely to fall when taking this drug. Tell your doctor if you feel dizzy or unsteady.

Other important information

Blood clot risk

Cancer and some cancer treatments can increase the risk of a blood clot. Contact the hospital straight away on the 24-hour number if you have any of these symptoms during or after treatment:

- throbbing pain or swelling in a leg or arm
- reddening of the skin in the area – if you have black or brown skin, this can be harder to notice, but the skin might become darker
- suddenly feeling breathless or coughing.

Always call **999** if you have:

- chest pain
- difficulty breathing.

A blood clot is serious, but it can be treated with drugs called anticoagulants. These thin the blood. Your doctor, nurse or pharmacist can give you more information about preventing and treating blood clots.

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Other medicines

Some medicines can affect how this treatment works or be harmful while you are having it. Always tell your cancer doctor, nurse or pharmacist about any drugs you are taking or planning to take, such as:

- medicines you have been prescribed
- medicines you buy in a shop, pharmacy or online
- vitamins or supplements
- herbal drugs and complementary or homeopathic therapies
- recreational drugs – for example, cannabis.

Vaccinations

Cancer doctors usually recommend that people with cancer have vaccinations for flu and coronavirus (covid). They may also recommend other vaccines, such as Shingrix® for shingles. These all help reduce your risk of serious illness from these infections. Most people can have these vaccines, including people with weak immune systems.

You should not have live vaccines if your immune system is weak. This includes if you are having or recently had chemotherapy, radiotherapy or other cancer treatments that affect your immune system. Live vaccines can make you unwell because they contain a very weak version of the illness they protect you against. There are several live vaccines, including the yellow fever vaccine.

It is important to ask your doctor, nurse or pharmacist for advice about having vaccinations. They can explain what vaccines are right for you and when it is best to have them.

Lactose

This treatment may contain lactose. If you have a lactose allergy or intolerance, ask your doctor, nurse or pharmacist for more information.

Sex

It is possible that small amounts of chemotherapy may be passed on through vaginal fluids or semen. If you have sex during this treatment, your cancer team will usually advise using condoms or a dental dam to protect your partner.

Contraception

Your doctor, nurse or pharmacist will advise you not to get pregnant or make someone pregnant while having this treatment and for some time afterwards. The drugs may harm a developing baby. You may be asked to take a pregnancy test before starting ivosidenib.

It is important to use effective contraception to prevent pregnancy. Follow the advice of your healthcare team about what types of contraception to use. Ivosidenib can affect how well hormonal contraception works so you will need to use a barrier method. Your healthcare team will tell you how long after treatment you should continue to use contraception.

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Fertility

Some cancer drugs can affect whether you can get pregnant or make someone pregnant. If you are worried about this, it is important to talk with your doctor before you start treatment.

Breastfeeding

You are advised not to breastfeed while having this treatment, or for some time after treatment ends. This is because the drugs could be passed to the baby through breast milk.

Your doctor, nurse or pharmacist can give you more information.

Medical and dental treatment

If you need medical treatment for any reason other than cancer, always tell the healthcare professional that you are having cancer treatment. Give them the contact details for your cancer doctor or cancer team so they can ask for advice.

If you have appointments with a dentist, always tell them you are having cancer treatment. Talk to your cancer team before you have any dental treatment.

About our information

This information has been written, revised and edited by Macmillan Cancer Support's Cancer Information Development team. It has been reviewed by expert medical and health professionals and people living with cancer.

References

Visit the electronic Medicines Compendium (eMC) to download a Patient Information Leaflet (PIL) for more detailed information. The leaflet lists all known side effects.

Cancer Information Development team

Author

Date reviewed

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Our cancer information meets the PIF TICK quality mark.

This means it is easy to use, up-to-date and based on the latest evidence. Learn more about how we produce our information.

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The language we use

We want everyone affected by cancer to feel our information is written for them.

We want our information to be as clear as possible. To do this, we try to:

- use plain English
- explain medical words
- use short sentences
- use illustrations to explain text
- structure the information clearly
- make sure important points are clear.

We use gender-inclusive language and talk to our readers as ‘you’ so that everyone feels included. Where clinically necessary we use the terms ‘men’ and ‘women’ or ‘male’ and ‘female’. For example, we do so when talking about parts of the body or mentioning statistics or research about who is affected.

You can read more about how we produce our information here.

How we can help

How to print a PDF from this website

Instructions on how you can make a PDF version of this page using the print function on your browser.

Macmillan Support Line

The Macmillan Support Line is a free and confidential service for people living with and affected by cancer. If you need to talk, we'll listen.

Call us free on 0808 808 0000

8am to 8pm, 7 days a week. You can talk to our cancer specialists about anything.

Email the Macmillan Support Line

Email the Macmillan Support Team if you have any questions.

Chat online

Available 8am to 7.45pm, 7 days a week. For confidential, real-time support if you don’t want to call.

Online Community

Chat online anonymously to others who understand what you are going through. Our community is available 24/7 and has dedicated forums where you can get advice and ask our experts.

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Find cancer support services in your area

What's going on near you? Find out about support groups, where to get information and how to get involved with Macmillan where you live.

Get help

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